

CLINICAL INTELLIGENCE TREND REPORT

GREEN: Routine Review

Patient ID: EGPatient ID: EGPeriod: March 15 to May 6, 2024Report Date: May 7, 2024Coverage: Bed: 915/1245h (73.5%)Chair: 670/1245h (53.8%)

Patient's monitoring period was less than 90 days; this report covers all available data. Decision-support summary derived from longitudinal vital sign trends; interpret in clinical context.

Clinical Alerting Thresholds

Very Low HR < 40 bpm

Very High HR > 110 bpm

Low HR < 45 bpm

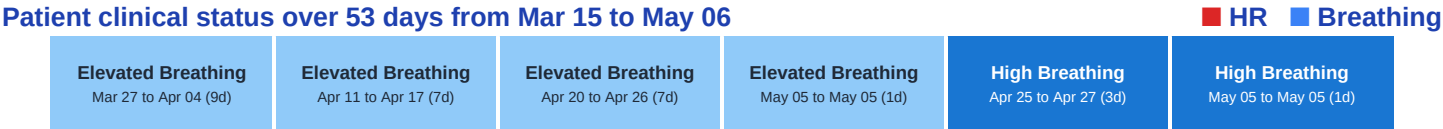
Elevated Breathing > 24 brpm

Elevated HR > 95 bpm

High Breathing > 30 brpm

High HR > 100 bpm

Very High Breathing > 40 brpm



Last 24 Hours

Last 24 Hours: NORMAL

30-Day Tier: GREEN

Vital	Avg	Min	Max
Heart Rate	60 bpm	48 bpm	79 bpm
Breathing	18 brpm	11 brpm	46 brpm

Last 24 hours: one very high breathing episode in the afternoon; one elevated breathing episode in the afternoon; one high breathing episode in the evening.

Episodic Events (last 24h)

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
May 05	1 PM to 2 PM	3h	3	Elevated Breathing	26	17	46	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 05	7 PM to 8 PM	1h	1	High Breathing (brief)	31	30	32	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	4 PM to 5 PM	1h	1	Very High Breathing (brief)	42	42	42	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF

39 episodic events Detected, spanning 45h total. Conditions: Elevated Breathing, High Breathing, Low Heart Rate, Very High Breathing. Priority grade: Low

Episodic Events	Total Hours	Highest Burden	Episodes/day	Coverage
39	45h	Elevated Breathing	<1	100%

Major Findings: Stable baseline

Date	Time Span	Duration	Episodes	Condition	Avg	Min	Max	Comment
Apr 25	4 PM to 5 PM	2h	2	High Breathing	35	23	46	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
May 05	7 PM to 8 PM	1h	1	High Breathing	31	30	32	Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
Mar 27	12 PM to 2 PM	14h	10	Elevated Breathing	27	19	40	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 11	2 PM to 3 PM	14h	12	Elevated Breathing	26	15	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
Apr 20	6 PM to 7 PM	7h	7	Elevated Breathing	27	16	44	Check SpO2 and lung sounds; assess for fluid overload or early infection
May 05	1 PM to 2 PM	2h	2	Elevated Breathing	26	17	46	Check SpO2 and lung sounds; assess for fluid overload or early infection

RR values outside physiologic range (6–60 breaths/min).

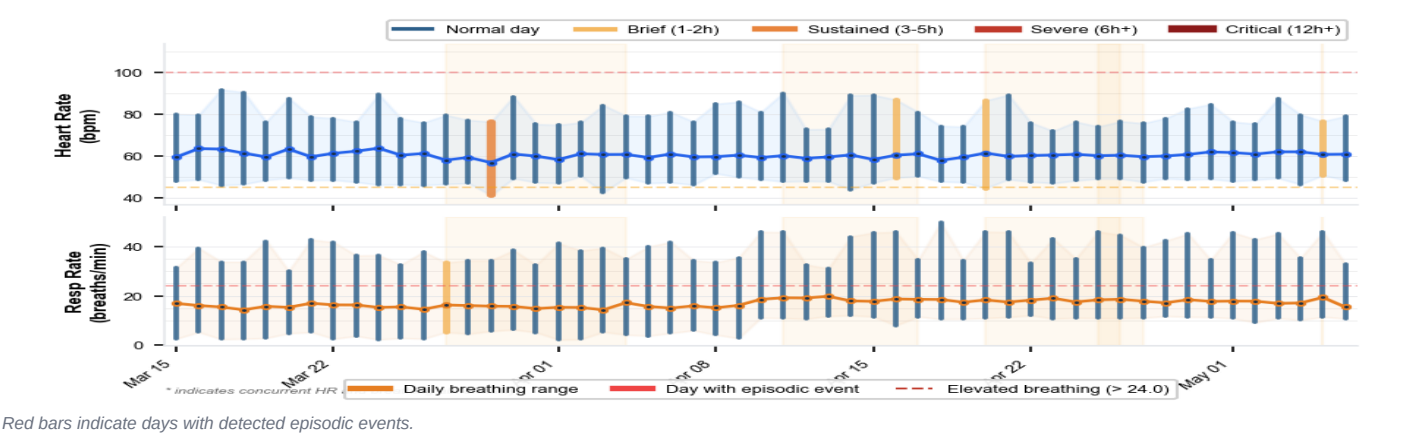
Suggested Clinical Review Actions

- Elevated Breathing (avg 27 brpm, peak 40 brpm): Check SpO2 and lung sounds; assess for fluid overload or early infection
- Low Heart Rate (avg 44 bpm, min 41 bpm): Check pulse and BP; review beta blockers and AV node agents; ECG if symptomatic

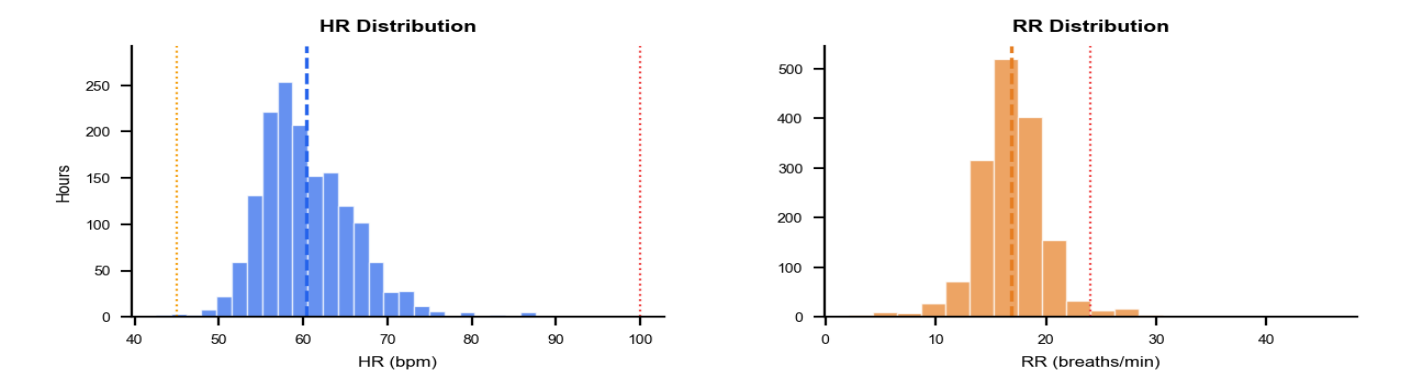
- Very High Breathing (avg 44 bpm, peak 46 bpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- High Breathing (avg 35 bpm, peak 46 bpm): Check SpO2, work of breathing, fever; consider pulmonary cause or CHF
- Overall trajectory shows 6 unstable phases with 17 bpm variability. Consider increasing monitoring frequency and lowering escalation threshold.

Clinical Pattern Observations:

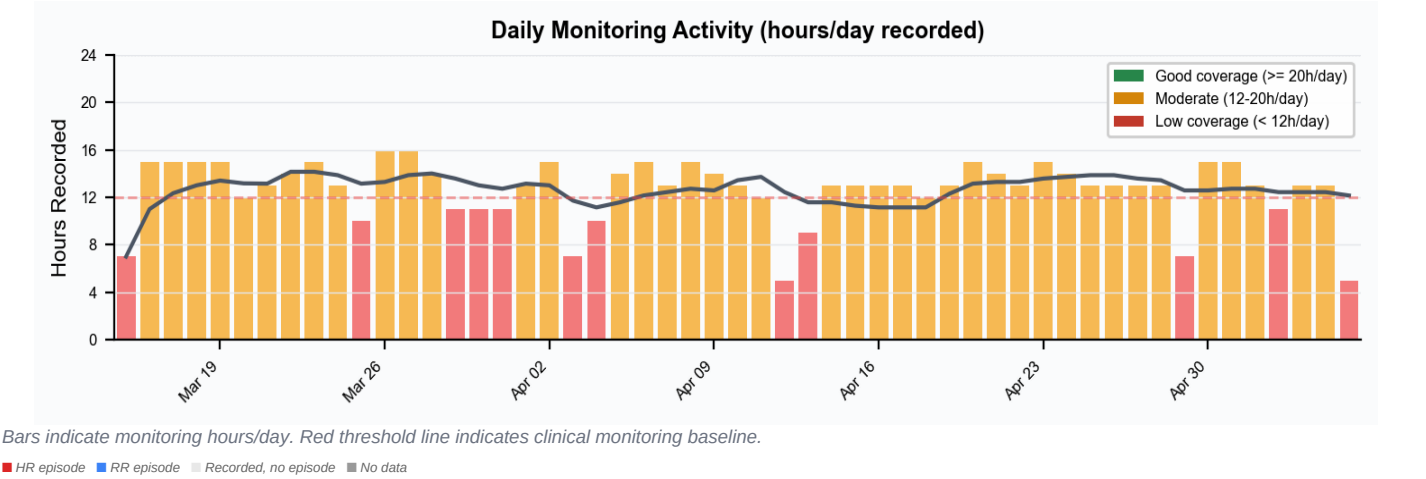
- 1. **Clustered pattern:** Episodic events on 5 of 53 days (9%).
- 2. **Daytime pattern:** Episodes concentrated during waking hours.



Vital Sign Distribution



Monitoring Activity



Episodic event: threshold exceeded continuously for ≥ 1 hour. Episodes separated by ≤ 1 hour of normal vitals are counted as the same event. Brief threshold crossings that do not sustain are not flagged.